

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 68 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: July 8, 2013 | Provider: Eleanor Park, MD — Internal Medicine | Doc ID: RFH-2013-0708-AWV

Annual Wellness Visit

Health Maintenance — Elevated Screening PSA Identified

Subjective

68-year-old man for annual visit. Feels well, remains tobacco-free (16 months). Mild urinary frequency and weaker stream over the past year; nocturia x2. No hematuria or dysuria. Diabetes and cardiac status stable.

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
130/80	60	15	36.6 C	98% RA	90.8 kg	29.6

Examination

Cardiac regular, no murmur. Lungs clear. Abdomen benign. Digital rectal exam: prostate mildly enlarged (≈35 g estimate), smooth, symmetric, no discrete nodule; no masses. Feet: monofilament intact bilaterally.

Screening Labs (drawn today; PSA resulted same day)

Test	Result	Units	Reference Range	Flag
PSA, total	7.8	ng/mL	< 4.0	H
Hemoglobin A1c	7.0	%	< 7.0 (goal)	
Creatinine	1.1	mg/dL	0.7 - 1.3	
eGFR (CKD-EPI)	74	mL/min/1.73 m2	> 60	
LDL cholesterol	68	mg/dL	< 70 (goal)	

Assessment

- Elevated PSA 7.8 ng/mL with lower urinary tract symptoms — concerning; requires urologic evaluation to distinguish BPH versus malignancy.
- Type 2 diabetes — at goal (A1c 7.0%).
- CAD / HTN / dyslipidemia — controlled, LDL at goal.
- Benign prostatic enlargement on exam.

Plan

- Urgent referral to Urology (Dr. Webb) for elevated PSA.
- Will repeat PSA to confirm prior to biopsy decision (urology to coordinate).
- Start tamsulosin 0.4 mg nightly for LUTS.
- Continue diabetes and cardiac regimen unchanged.
- Colorectal cancer screening up to date (colonoscopy 2011 — normal, per records).